NCCP 00689 · VERSION 4

Your treatment, explained

First-line extensive-stage small-cell lung cancer · NCCP 00689 v4 · Document revised 4 November 2026

> **Your personal plan:** this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Atezolizumab + carboplatin + etoposide

21-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact your oncology team immediately for fever or shivering, sudden illness even without a fever, new or worse breathlessness, diarrhoea, severe rash or unusual weakness. Severe chest pain, collapse, confusion or major breathing difficulty need emergency help. Show your immunotherapy alert card; immune effects can begin after treatment ends. Report new muscle weakness, drooping eyelids or difficulty swallowing immediately, especially with chest symptoms or breathlessness; immune effects on muscles, nerves and the heart can occur together.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: hse · nccp · smpc

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- Induction and maintenance
- Your 21-day timeline
- Baseline and cycle-specific checks

Whole treatment, scans and checks
Tablets, visits and practical planning
Understanding this treatment
Before the first treatment
What your tests and scans are checking
Side effects: what they mean and what to do
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Living with treatment
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Your route and capsule plan
Supportive medicines
Understanding the evidence
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UNDERSTAND YOUR TREATMENT

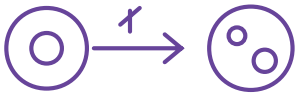
Three medicines. Different jobs.

To control extensive-stage small-cell lung cancer, relieve symptoms and help people live longer. The balance of benefit, side effects and time in treatment should be reviewed with you.

IMMUNOTHERAPY

Atezolizumab

Tecentriq



Blocks the PD-L1 immune brake

Atezolizumab is an antibody against PD-L1, a signal that can reduce an immune cell’s activity. Blocking that signal can help immune cells attack cancer. It works differently from chemotherapy. The same immune activation can inflame healthy organs, so new symptoms need assessment even when they look like ordinary treatment tiredness or a minor infection. IV and SC formulations use different doses but both carry immune risks.

Immune cell → PD-L1 blocked → Cancer cell

What to watch for

Report new cough, breathlessness, diarrhoea, rash, severe headache, weakness, thirst, frequent urination or chest symptoms promptly. Effects can appear during treatment or after the final dose.

Sources: nccp · smpc

CHEMOTHERAPY

Carboplatin

Platinum chemotherapy



Damages cancer-cell DNA

Carboplatin forms links in DNA that interfere with copying and repair. This helps stop dividing cancer cells but also affects healthy tissues such as bone marrow. Kidney function is used to calculate its dose. It is given during the four-cycle induction phase and stops before atezolizumab-only maintenance.

DNA strands → Platinum links → Copying disrupted

What to watch for

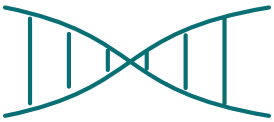
Report fever, shivering, unexpected bruising, bleeding or persistent vomiting. Tell the nurse immediately about flushing, itching, chest tightness, breathing changes or pain around the drip.

Sources: nccp

CHEMOTHERAPY

Etoposide

Topoisomerase II chemotherapy



Interferes with DNA repair

Etoposide interferes with topoisomerase II, an enzyme that helps manage DNA during copying. DNA damage can then accumulate and stop cell division. Healthy rapidly renewing tissues can also be affected, explaining low blood counts, hair loss and mouth or bowel problems. Oral absorption differs from IV delivery; the prescribed oral dose must not be inferred from the IV dose.

DNA copying → Repair interrupted → Cell division slows

What to watch for

During an infusion, report dizziness, breathing changes, rash or pain at the drip. At home report fever, mouth ulcers, diarrhoea or poor intake. If capsules are prescribed, follow the exact labelled doses and contact the team about a missed or vomited dose rather than replacing it yourself.

Sources: nccp · etoposide-smpc

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

THE BIGGER TREATMENT PLAN

Induction → review → maintenance

THREE MEDICINES

Induction · up to four cycles



Day 1 all three medicines; etoposide also Days 2–3. A cycle is 21 days. Confirm the exact IV/SC/oral route plan.

ATEZOLIZUMAB ONLY

Maintenance · after induction



Chemotherapy stops. Atezolizumab continues every 21 days while appropriate; there is no fixed total number here. Review benefit, symptoms and supportive prescriptions.

Maintenance means ongoing treatment, not treatment without side effects. Your plan can change after discussion.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Treatment checks across the course

Relative stages, not equally spaced dates. No patient dates are needed; agree who arranges and reviews checks.

Check	Before starting	Induction Day 1	Induction Days 2–21	Maintenance review	After treatment
Bloods and hormones	FBC, liver/kidney, glucose, thyroid, hepatitis B/C and GFR baseline	Review results and current symptoms	FBC, liver/kidney and glucose each cycle; thyroid every 3–6 weeks	Continue cycle bloods and thyroid checks	Follow unresolved problems and the team's plan
Treatment and routes	Agree IV/SC atezolizumab and IV versus IV/oral etoposide	All three medicines in the source sequence	Etoposide Days 2–3; next cycle usually after 21 days	Carboplatin and etoposide stop after up to four cycles	Agree follow-up and next treatment decisions

Check	Before starting	Induction Day 1	Induction Days 2–21	Maintenance review	After treatment
Support medicines	Prepare one reconciled day-by-day anti-sickness plan	Carboplatin-specific high-risk cover	Account for etoposide Days 2–3 and two days afterwards	No automatic continuation of chemotherapy prophylaxis	Continue only individually prescribed medicines
Immune effects and cancer review	Baseline breathing, bowel, skin and endocrine symptoms; alert card	Report administration reactions immediately	Report new cough, diarrhoea, rash or unusual weakness promptly	Review benefit and symptoms; scans per consultant	Delayed immune effects remain possible

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00689 v4 · official regimen

THE WHOLE TREATMENT JOURNEY

Atezolizumab + carboplatin + etoposide: treatment and checks

First-line extensive-stage small-cell lung cancer

Induction: atezolizumab and carboplatin on Day 1, with etoposide on Days 1–3, repeated every 21 days for up to four cycles. Maintenance: carboplatin and etoposide stop; atezolizumab alone continues every 21 days while appropriate. The fourth cycle ends about 12 weeks after starting if there are no delays; maintenance has no fixed total duration in this regimen.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before starting	Induction Day 1	Induction Days 2–21	Maintenance review	After treatment
Bloods and hormones NCCP 00689	FBC, liver/kidney, glucose, thyroid, hepatitis B/C and GFR baseline	Review results and current symptoms	FBC, liver/kidney and glucose each cycle; thyroid every 3–6 weeks	Continue cycle bloods and thyroid checks	Follow unresolved problems and the team's plan
Treatment and routes NCCP 00689	Agree IV/SC atezolizumab and IV versus IV/oral etoposide	All three medicines in the source sequence	Etoposide Days 2–3; next cycle usually after 21 days	Carboplatin and etoposide stop after up to four cycles	Agree follow-up and next treatment decisions
Support medicines NCCP 00689	Prepare one reconciled day-by-day anti-sickness plan	Carboplatin-specific high-risk cover	Account for etoposide Days 2–3 and two days afterwards	No automatic continuation of chemotherapy prophylaxis	Continue only individually prescribed medicines
Immune effects and cancer review NCCP 00689	Baseline breathing, bowel, skin and endocrine symptoms; alert card	Report administration reactions immediately	Report new cough, diarrhoea, rash or unusual weakness promptly	Review benefit and symptoms; scans per consultant	Delayed immune effects remain possible

What each check is for

1

Before treatment

Before first dose

Confirm suitability, phase and routes

Ask / act: Review results, contraindications, medicines and explicit route prescriptions.

Sources: NCCP 00689

2

Phase change

After up to four induction cycles

Chemotherapy ends; immune treatment may continue

Ask / act: Confirm atezolizumab schedule and revise anti-sickness medicines without stopping required hormone replacement.

Sources: NCCP 00689

3

After treatment

Completion or change of plan

Delayed effects and follow-up

Ask / act: Give follow-up arrangements, alert-card advice and urgent contact details.

Sources: NCCP 00689

At the next review, ask

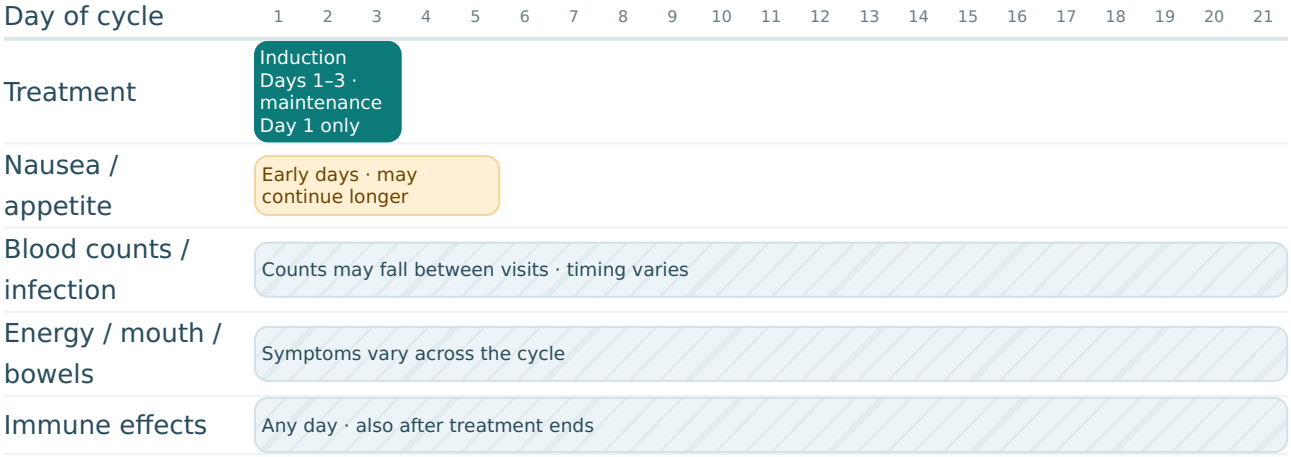
- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Induction: atezolizumab and carboplatin on Day 1, with etoposide on Days 1–3, repeated every 21 days for up to four cycles. Maintenance: carboplatin and etoposide stop; atezolizumab alone continues every 21 days while appropriate. The fourth cycle ends about 12 weeks after starting if there are no delays; maintenance has no fixed total duration in this regimen.



About 12 weeks of induction if on schedule: Four 21-day cycles include both treatment days and the interval before the next cycle; delays can extend the course.

Day 2–3 route matters: These days are IV visits or prescribed oral etoposide twice daily. They are not days without treatment during induction.

No fixed total maintenance length: Atezolizumab continues while the team judges it appropriate, until progression or unacceptable toxicity under this NCCP regimen. The plan is reviewed with you.

Checks continue after chemotherapy: Maintenance still requires cycle bloods, thyroid tests every 3–6 weeks and ongoing symptom review. Immune effects may begin after the last atezolizumab dose.

Why these bands stay broad: This reference describes a 21-day cycle. Symptoms do not follow a fixed safe/unsafe calendar, and improving symptoms do not prove blood counts have recovered. Your written treatment plan sets dates and route. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.

There are no guaranteed “safe days”. These materials explain the three-week NCCP 00689 regimen and its explicitly prescribed route alternatives. They are educational and cannot establish treatment suitability, calculate doses or replace the team’s written prescription.

Read the whole cycle as text

Day 1: Day 1: treatment and checks

The team checks symptoms and blood tests, then gives atezolizumab, carboplatin and etoposide during induction. Atezolizumab may be IV or SC. Tell staff immediately if you feel unwell during treatment. Before leaving, confirm whether Days 2–3 mean IV visits or oral etoposide at home, and check the anti-sickness plan.

Day 2–3: Days 2–3: etoposide during induction

You either attend for etoposide drips or take the separately prescribed etoposide capsules twice daily, according to your confirmed plan. Capsules are taken on an empty stomach. Maintenance does not include etoposide. Do not switch between the IV and oral schedules or infer capsule amounts. Contact the team about missed or vomited doses.

Day 4–7: After the chemotherapy days

Sickness, reduced appetite, fatigue or bowel changes may occur. Take the anti-sickness medicines on your written schedule, including later days. The team should check coverage after the last chemotherapy day. Ask for advice if vomiting continues or you cannot drink. Report new diarrhoea promptly because immune bowel inflammation is also possible.

Day 8–14: Keep checking how you feel

Blood counts may fall between visits. The day number does not tell you whether your counts are safe. Hair loss, mouth soreness or fatigue can affect daily life; ask for symptom support. Use the urgent contact plan for fever, shivers or sudden illness, including when your temperature is normal.

Day 15–21: Prepare for the next review

Some symptoms may settle while others persist or appear for the first time. The next dose depends on your review and tests. Around the end of induction, the team discusses whether to move to atezolizumab-only maintenance. Bring your symptom record and medicines list. Confirm the next date and which phase and routes are planned.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Appointments and time in the unit

Day 1 order: atezolizumab, carboplatin, then etoposide. Atezolizumab IV first infusion is 60 minutes, later 30 minutes if tolerated; SC injection takes about 7 minutes. Carboplatin infusion is 30 minutes and etoposide IV is 60 minutes. Total unit time also includes checks, preparation and observation. SC atezolizumab does not remove the chemotherapy drips during induction.



STEP 2

Your written formulation plan

Confirm your written route plan. Atezolizumab may be a drip (IV) or an injection under the skin of the thigh (SC). Etoposide may be IV on Days 1–3, or IV on Day 1 with prescribed capsules twice daily on Days 2–3. These schedules use different doses; do not change route or convert doses yourself. A separately prescribed four-week IV atezolizumab maintenance plan uses NCCP 00593 and is outside this 21-day calendar.



STEP 3

Travel, support and medicines

Induction usually involves treatment on three consecutive days or one IV day plus prescribed capsules at home. Ask about transport, work, caring responsibilities and who to call about capsules or symptoms. Carry the immunotherapy alert card.



STEP 4

Tests and maintenance

FBC, liver, renal and glucose profile before every cycle; thyroid tests every 3–6 weeks. Scans and other disease assessment follow the consultant plan. Continue symptom review during maintenance and after atezolizumab stops because immune effects may be delayed. Confirm which team books each test and who reviews it before treatment.

Open practical treatment guide · **Printable practical guide**

Sources: NCCP 00689 v4 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Day 1 carboplatin anti-sickness plan

Carboplatin has a high sickness risk. Your prescription usually includes several anti-sickness medicines with some doses on later days. Follow the written plan even if you initially feel well; contact the team if it is not effective.

2 Days 2–3 etoposide and the days afterwards

The team should account for each chemotherapy day and for two days after the last dose. The later-day plan may overlap the medicines already started on Day 1, so obtain one clear schedule and do not add duplicate doses. Confirm the plan for oral etoposide separately.

3 Maintenance support changes

When only atezolizumab remains, routine chemotherapy anti-sickness steroids and other prophylaxis are not automatically continued. Keep any individual rescue or hormone-replacement plan and the immunotherapy alert card. Do not stop prescribed long-term steroids yourself.

Sources: nccp · ncis

Understanding this treatment

A three-medicine induction phase, then atezolizumab-only maintenance.

Your treatment aim

To control extensive-stage small-cell lung cancer, relieve symptoms and help people live longer. The balance of benefit, side effects and time in treatment should be reviewed with you.

Sources: nccp

How the phases fit together

Induction: atezolizumab and carboplatin on Day 1, with etoposide on Days 1–3, repeated every 21 days for up to four cycles. Maintenance: carboplatin and etoposide stop; atezolizumab alone continues every 21 days while appropriate. The fourth cycle ends about 12 weeks after starting if there are no delays; maintenance has no fixed total duration in this regimen.

Sources: nccp

Confirm your route plan

Confirm your written route plan. Atezolizumab may be a drip (IV) or an injection under the skin of the thigh (SC). Etoposide may be IV on Days 1–3, or IV on Day 1 with prescribed capsules twice daily on Days 2–3. These schedules use different doses; do not change route or convert doses yourself. A separately prescribed four-week IV atezolizumab maintenance plan uses NCCP 00593 and is outside this 21-day calendar.

Sources: nccp

Reviewing benefit and choices

Symptoms, scans and how treatment affects your daily life all matter. Ask when benefit will be reviewed and what would prompt a delay, change or stop. There is no universal scan calendar in this NCCP regimen. You can revisit consent and discuss supportive care at any point.

Sources: nccp

TREATMENT REFERENCE · 02

Before the first treatment

Use this as a conversation checklist. Your own hospital’s instructions and prescription take priority.

Tell the team your starting point

Describe your usual breathing, bowel pattern, energy, skin symptoms and ability to manage daily tasks. Mention autoimmune illness, organ transplant, previous lung inflammation, infections and all regular medicines. Changes are easier to recognise when the team knows what is normal for you. Do not minimise symptoms because you are worried treatment will be delayed.

Sources: nccp

Make the first week manageable

Have the oncology and out-of-hours numbers available, a working thermometer, your medicine schedule and a way to get urgent help. Ask who to ring if you miss a supportive medicine. Plan transport and practical help if needed. Bring your questions and, if you wish, someone you trust to listen and take notes.

Sources: nccp

During the infusion visit

Expect symptom and blood-test review before medicines are released. Chemotherapy is given through agreed venous access; atezolizumab may be a drip or a thigh injection. Tell staff immediately about pain or swelling around the treatment site, itching, flushing, dizziness or breathing changes. Preparation and observation add to the visit time.

Sources: nccp

TREATMENT REFERENCE · 03

What your tests and scans are checking

Different checks answer different questions: whether treatment is safe today, whether a symptom needs investigation, and whether the cancer is responding.

Blood counts: three separate jobs

Neutrophils help fight infection; a low level can make infection dangerous. Haemoglobin carries oxygen; a low level can contribute to tiredness and breathlessness. Platelets help stop bleeding; low levels can cause bruising or bleeding. Feeling well does not prove your counts are normal, and a previous normal result does not rule out a new problem.

Sources: nccp

Kidney, liver and hormone checks

Kidney function helps the team calculate carboplatin and review etoposide dosing. Liver and kidney tests can also reveal immune inflammation or other problems before symptoms develop. Glucose is checked each cycle and thyroid function every three to six weeks. Fatigue, weakness, thirst, dizziness or new headaches may need extra tests; do not assume that they are ordinary treatment effects.

Sources: nccp

Scans and the treatment decision

Your team sets the scan schedule and compares scans with symptoms and examination findings. Stable disease can be a worthwhile result. New symptoms need review even before the next scan. An unusual scan pattern sometimes needs clarification; do not assume that apparent growth is simply an immune response or that feeling worse means treatment is working.

Sources: nccp

Why a delay or a change can be appropriate

A pause may allow recovery or investigation. Chemotherapy doses can sometimes be reduced. Atezolizumab is generally held or stopped for relevant toxicity rather than dose-reduced. Ask which drug is changing and why; stopping one drug does not automatically mean stopping every part of the plan. Long-term treatment decisions belong in a discussion about benefit, risk and what matters to you.

Sources: nccp

Side effects: what they mean and what to do

These are selected possibilities, not a prediction or a complete medicine leaflet. You do not need to work out the cause before asking for help.

Nausea, vomiting and appetite change

Medicines can stimulate sickness pathways in the gut and brain. Take preventive anti-sickness medicines as prescribed, even when you initially feel well. Smaller meals and frequent sips may be easier. Report poor control early so the plan can be adjusted. Persistent vomiting, inability to keep fluids down or much less urine need prompt advice. Do not wait until you are very dehydrated.

Sources: nccp

Fatigue: common, but not always “just treatment”

Fatigue may reflect treatment, disturbed sleep, reduced intake, anaemia or another illness. Balance rest with manageable movement and prioritise activities that matter to you. A sudden change, severe weakness, dizziness, breathlessness or inability to manage basic tasks needs assessment; hormone problems and other complications can resemble ordinary tiredness. Tell the team how fatigue affects daily life, not just that you feel tired.

Sources: nccp

Mouth soreness, ulcers and taste

Treatment can injure the lining of the mouth. Use a soft toothbrush, gentle mouth care and the rinses recommended by your team. Choose softer foods and avoid foods that sting. Ask for pain relief early. Report ulcers, white patches, painful swallowing or reduced intake; difficulty drinking or fever needs urgent advice. Arrange dental care with the oncology team before procedures.

Sources: nccp

Diarrhoea or abdominal pain

Chemotherapy can upset the bowel, and atezolizumab can cause bowel inflammation. Report new diarrhoea promptly, including stools above your usual pattern, blood, mucus, pain or night-time symptoms. Do not assume that it is ordinary chemotherapy upset. Severe pain, bleeding or dehydration needs urgent assessment.

Sources: nccp

Constipation

Anti-sickness drugs, opioid painkillers, less food and less movement can slow the bowel. Ask about a bowel plan if you are prone to constipation or take opioids. Keep track of your usual pattern. Increasing abdominal pain, vomiting, a swollen abdomen or inability to pass wind needs prompt assessment rather than simply increasing laxatives.

Sources: nccp

Bruising, bleeding or breathlessness

Chemotherapy may reduce platelets or red cells. Report unexpected bruises, nosebleeds, bleeding gums or increasing breathlessness. Heavy bleeding, black stools with weakness, severe chest pain or severe breathing difficulty need emergency help. Do not start iron or alter blood thinners without review: the cause and treatment need to be established.

Sources: nccp

Skin, hair and nerve changes

Etoposide can cause hair loss, which may affect scalp and body hair. Ask about practical support before it begins. Report a new or spreading rash, tingling, numbness or weakness. Blisters, peeling skin or sores involving eyes or mouth need urgent assessment because immune skin reactions can be serious. Use gentle skin care and ask about suitable sun protection.

Sources: nccp

TREATMENT REFERENCE · 05

Atezolizumab: symptoms that deserve special attention

Immune treatment can cause inflammation in healthy organs, during treatment or after it ends. Keep your immunotherapy alert card and tell other healthcare professionals you have received atezolizumab.

Lungs, bowel, liver and kidneys

New cough or breathlessness, diarrhoea, yellow skin or eyes, dark urine, reduced urine or swelling can signal inflammation. These symptoms also have other causes, including infection and cancer. Contact the team promptly for assessment rather than trying to identify the organ or diagnosis yourself.

Sources: nccp

Hormones, heart and nervous system

Report unusual weakness, persistent headache, dizziness, marked thirst, frequent urination, palpitations, muscle weakness or changes in vision. Severe chest pain, collapse, confusion, seizures or major breathing difficulty require emergency care. Some hormone effects need ongoing replacement treatment even after immunotherapy stops.

Sources: nccp

Why early reporting matters

The team may pause treatment and investigate. Some reactions need steroids or other immune-suppressing treatment; others need hormone replacement. These are different from the short planned steroid course around chemotherapy. Do not start leftover steroids, change their dose or stop a prescribed course yourself. Carry the treatment name when attending emergency care.

Sources: nccp

TREATMENT REFERENCE · 06

Living with treatment

The aim is a plan that fits your life as well as your treatment. Bring practical concerns to the team early.

Food, fluids and weight

Aim for enough food and fluid to maintain strength, using foods you tolerate. Tell the team about weight loss, swallowing problems or difficulty shopping or cooking; ask for dietitian support. A fixed fluid target is not right for everyone, particularly with heart or kidney disease. “Anti-cancer” diets, fasting and high-dose supplements are not substitutes for treatment and can create nutritional or interaction problems.

Sources: nccp

Movement, work and rest

Try activity that feels manageable and increase or reduce it according to symptoms. Short walks or simple daily tasks may be more realistic than your usual exercise. Discuss work adjustments and travel plans. Do not drive if dizzy, unusually tired or affected by sedating medicines. Sudden deterioration should prompt assessment, not an attempt to push through exercise.

Sources: nccp

Infection precautions without isolation

Wash hands, practise food hygiene and avoid close contact with people who are unwell. Ask about vaccinations and their timing; some vaccines are unsuitable during treatment. You can usually still spend time with family. Keep your urgent contact plan available: precautions reduce risk but do not make fever or sudden illness safe to watch at home.

Sources: nccp

Intimacy, fertility and pregnancy

Discuss fertility preservation before treatment if relevant. Treatment may harm a developing baby; ask for a written contraception plan covering all three medicines and the period after the last dose. Do not breastfeed during treatment; confirm when it would be safe to restart. Tell the team promptly about a possible pregnancy. Ask about barrier protection and handling body fluids under your hospital’s advice.

Sources: nccp

Emotional support and choices

It is reasonable to feel worried, angry, uncertain or overloaded. Ask for psycho-oncology, counselling, social work or family support. You can request information in smaller pieces, an interpreter or a written summary. Tell the team what you most want to preserve in daily life. You can revisit treatment decisions and ask for more time or another explanation.

Sources: nccp

TREATMENT REFERENCE · 07

A short treatment dictionary

Terms you may hear in clinic.

Cycle · induction · maintenance

A cycle is a planned treatment-and-interval period. Induction is the initial combination phase. Maintenance is ongoing treatment after that phase; it does not mean side-effect-free treatment.

Sources: nccp

Response · stable disease · progression

Response means the cancer has reduced by the assessment used. Stable disease means it has not changed enough to meet response or progression criteria. Progression means growth or spread meeting the assessment criteria. Your team interprets these alongside how you are feeling.

Sources: nccp

Neutropenia · anaemia · thrombocytopenia

Neutropenia means low infection-fighting neutrophils. Anaemia means low haemoglobin. Thrombocytopenia means low platelets. These are different results with different implications; one normal number does not establish that all treatment risks are low.

Sources: nccp

Immune-related · colitis · pneumonitis

Immune-related effects come from immune activity against healthy tissue. Colitis means inflammation of the colon; pneumonitis means inflammation in the lungs. A symptom alone cannot establish either diagnosis.

Sources: nccp

TREATMENT REFERENCE · 08

Your route and capsule plan

The NCCP source lists four combinations of routes. Your team must select and prescribe one clearly.

IV or SC atezolizumab

The IV formulation runs through a drip; the SC formulation is injected under the skin of the thigh. Their doses differ. The nurse checks the correct formulation and injection site. Both can cause immune and administration reactions; tell staff about pain, swelling, rash, dizziness or breathing changes.

Sources: nccp

Etoposide Days 2–3

The standard IV schedule has a drip on all three days. The oral alternative has IV etoposide on Day 1 and capsules twice daily on Days 2–3. Follow the exact capsule label and empty-stomach instructions. Do not calculate a replacement oral dose from a drip dose, split a capsule or double a missed dose. Ask the team what to do after a missed or vomited dose.

Sources: nccp

Handling and supply

Keep capsules in their original labelled packaging, away from children. Follow the pharmacy’s handling and disposal advice; ask what carers should do if a capsule is damaged. Check the full supply and both daily doses before leaving the unit. Do not use remaining capsules for a later cycle unless the team confirms the new prescription.

Sources: nccp

Changing phase

All carboplatin and etoposide finish when induction ends. Atezolizumab-only maintenance in this guide is every three weeks. If your team selects four-week IV maintenance, obtain the separate written schedule; this guide must not be used to convert it.

Sources: nccp

Why this treatment is offered

In a randomised trial in previously untreated extensive-stage small-cell lung cancer, adding atezolizumab to carboplatin and IV etoposide improved survival and delayed progression on average. This cannot predict one person’s benefit.

In a randomised trial in previously untreated extensive-stage small-cell lung cancer, adding atezolizumab to carboplatin and IV etoposide improved survival and delayed progression on average. This cannot predict one person’s benefit.

Sources: trial-v08423 · trial-primary

PREPARE FOR THE CONVERSATION

Questions worth asking

What is treatment trying to achieve?

Ask how likely this plan is to help control your cancer and symptoms, what alternatives exist, and when you will review benefit.

Which routes am I receiving?

Confirm IV or SC atezolizumab and IV etoposide on Days 1–3 versus IV Day 1 with oral etoposide twice daily Days 2–3. Ask for a written plan.

How long will induction take?

Four 21-day cycles occupy about 12 weeks if there are no delays. Chemotherapy then stops; atezolizumab maintenance may continue while appropriate.

Who arranges my tests and scans?

Confirm blood tests before each cycle, thyroid checks every 3–6 weeks and the consultant’s scan plan. Ask who checks and explains the results.

What changes when maintenance starts?

Carboplatin and etoposide stop. The visit pattern and anti-sickness plan change, but immune symptom reporting and blood/hormone checks continue.

Who do I call outside clinic hours?

Keep your oncology emergency number and written temperature/symptom instructions. Do not wait for the next appointment when unwell; show your immunotherapy alert card.

> **Try explaining the plan back:** Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00689 v4. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00689 v4 · official regimen
- NCCP NCIS Medical Oncology antiemetics V8 (2025)
- IMpower133 updated overall survival (2021)
- IMpower133 primary randomised analysis (2018)

- EMA Tecentriq current product information
- HSE chemotherapy safety and urgent symptoms
- Etoposide capsules · manufacturer product information and shared etoposide safety
- IMbrella A / IMpower133 exploratory long-term follow-up (2024)

SACTCheck · Developed by Dr Paul O’Brien · Sources checked 2026-10-29.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6882/689_V4_Atezolizumab_Etoposide_CARBOplatin.pdf

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf

https://pubmed.ncbi.nlm.nih.gov/33439693/

https://pubmed.ncbi.nlm.nih.gov/30280641/

https://www.ema.europa.eu/en/documents/product-information/tecentriq-epar-product-information_en.pdf

https://www2.hse.ie/conditions/chemotherapy/side-effects/

https://www.medicines.org.uk/emc/product/10533/smpc

https://pubmed.ncbi.nlm.nih.gov/39306923/